

Medical Brain Drain: Causes and Implications in Nigeria's Health Care System

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Abstract:

The global migration of doctors and nurses from developing nations to developed countries generates serious deficits in the underdeveloped countries, demonstrating one of the means through which global capitalism evacuates dynamics of surplus extraction from the periphery to global wealth centers. Medical brain drain has become a global growing burden because it deepens the already deprecated healthcare system in poor countries and widens the gap in health inequities worldwide. This study, therefore, tends to examine the causes and implications of the medical brain drain in Nigeria's health care system. The study relied on secondary sources and data were sourced from journals, magazines, articles, textbooks, and personal experience. Dependency theory also known as underdevelopment theory propounded by Raul Prebisch in the 1950s was used in analyzing the reason behind the migration of Nigerian medical professionals to developed countries. However, thematic and content analysis was used as a methodology that guides the research. The paper identified the major causes of medical brain drain in Nigeria to include "push factors" which are associated with the country of origin and "pull factors" which are attributed to the countries of destination. The paper also found that medical brain drain has affected Nigeria's health care system negatively in different ways including ineffective service delivery, economic backwardness, to just a few. Therefore, the research recommends amongst other things that, Nigeria should address the structural, political, and economic problems that lead to the brain drain and show foresight and commitment to improving the living conditions to succeed in effecting a brain gain.

KEYWORDS: Medical brain, causes, implication, Health Care System.



INTRODUCTION

The term "Brain Drain" was coined by the British Royal Society to describe the outflow of scientists and technicians to the United States and Canada in the 1950s and early 1960s. By the 1970s brain drain came to be associated with the flow of skilled individuals from the developing world to Western Europe and

North America [1]. On the other hand, the Medical brain drain is defined as the migration of health professionals from developing countries to developed ones in search of a better standard of living and life quality, higher salaries, access to advanced technology, and more stable political atmosphere [2]. Brain drain

may be within the country but in most cases refers to the cross-border or international migration of highly skilled professionals. The transnational migration of doctors and nurses generates serious deficits in the developing world, illustrating one of the ways that global capitalism sets up dynamics of surplus extraction from the developing world to civilized countries.

The phenomenon of "Brain drain" attracted the interest of researchers and provoked controversy starting from its origins (in the 1950s) until the present. Although the highly skilled migration has its roots in prehistoric times, significant outflows were reported from the third decade of the 20th century (the inter-war period is defined as a forced emigration from Europe to the United States – German intellectuals requesting asylum) [3]. The medical brain drain has become a global growing burden due to its consequence on both the donor and the destination countries. It deepens the already deprecated healthcare resources in poor nations and expands the gap in health unfairness globally [4]. The migration of health specialists in Africa deserves critical attention due to its unfavorable effects on the healthcare system (HCS) of developing nations, which indirectly affects population health outcomes and creates greater inequity among vulnerable populations.

In Africa alone, where health is a need and the problem is greatest, around 23,000 medical doctors emigrate annually [5]. In the case of Nigeria, medical brain drain is a major challenge confronting its health system, leading to a dramatic decline in the number of medical professionals in the country. According to the Organisation for Economic Co-operation and Development, Nigeria is among the three leading African contributors of foreign-born physicians. It has been estimated that — Each week, a minimum of 12 Nigerian specialists are employed in the United Kingdom; more than 4,000 are already working in the United States, while Canada ceaselessly attracts medical experts from Nigeria. The Association of Nigerian Physicians in America has more than 4,000 members; data from the U.K. reveals that more than 5,000 Nigerian consultants are working there; and the Canadian Medical Association Masterfile has registered a quadrupling of Nigerian doctors practicing in the country over a decade, from 176 in 2008 to 568 in 2018 [6&7]. Other top countries that evacuated Nigerian medical doctors include Saudi Arabia, Kuwait, and the United Arab Emirates and there is no end in sight. The 2017 survey confirmed that 9 out of 10 medics in Nigeria needed to depart the country as a result of poor working conditions and inadequate salary. Another study revealed that high figures are registering daily to take foreign medical exams enabling them to work abroad [6&7]. This of course has caused a lot of problems to Nigeria's health care system. So what are the causes of all this appalling and debilitating situation of Nigeria's health care system? What are the forces that influence medical experts to migrate from their home country to abroad? This paper, therefore, tends to examine these questions after which antidotes to the problem will be recommended.

LITERATURE REVIEW

The concept of brain drain has its root in prehistoric times and the phenomenon attracted the interest of researchers and provoke controversy from the 1950s until the present. The significant outflows were reported from the third decade of the

20th century (the inter-war period is defined as a forced emigration from Europe to the United States – German intellectuals requesting asylum [2]). The medical brain drain and its impact on the developing countries attracted interest and reactions amidst the research community. In Africa alone, where health is a need and the problem is greatest, around 23,000 medical doctors emigrate annually [5]. Information from South African medical schools suggests that a third to a half of its graduates immigrate to the developed world [8]. The loss of nurses has been even more extreme in many African countries. For example, more than 15000 Filipino nurses and Zimbabweans nurses work abroad. A recent report from the United Kingdom estimated that 31% of its doctors and 13% of its nurses are born overseas; in London, the figures are 23% and 47% respectively [9,10,11].

The cost implications of all these migrations are critical. For example, the United Nations Commission for Trade and Development has calculated that each migrating African medical skilled embodies a losing of \$184000 to Africa. Paradoxically, Africa pays \$4billions per annum on the remunerations of 100,000 foreign experts. In an example of a medical brain drain within the country, Kenya estimates that 600 doctors work in public hospitals out of more than 5000 registered; the rest have moved abroad or are working in the private sector. Brain drain also occurs when health workers end up working outside the health sector or unskilled labor in the country they move to [12,13,14,15]. In Nigeria, medical brain drain is one of the major challenge confronting its health system, leading to a dramatic decline in the number of medical professionals in the country. According to the Organisation for Economic Co-operation and Development, Nigeria is among the three leading African sources of foreign-born physicians with more than 4,000 and 5,000 working in America and United Kingdom respectively [6&7]. Other top countries that evacuated Nigerian medical doctors include Saudi Arabia, Kuwait, and the United Arab Emirates and there is no end in sight.

The implication of all these migrations on Nigeria is of serious concern which needs urgent attention before to be halted rectified, and become history. Expressing his concern over the implications of medical brain drain in Nigeria, Ibrahim Adeoti Katibi, professor and the Dean College of Health Sciences, University of Ilorin Teaching Hospital (UIITH), has disclosed that Nigeria spends over \$1 billion annually on medical tourism. Katibi, who made the disclosure in Ilorin at the 36th annual general meeting and scientific conference of Association of Resident Doctors equally revealed that 500 Nigerians go abroad monthly to seek medical intervention. According to him, Nigeria has competence and expertise but lacks adequate facilities. He said over \$1billion is spent annually on medical care abroad and 500 Nigerians go to India, Europe, and other countries to receive medical attention. If we invested such an amount of money in our health sector; it will go a long way in improving health care service in Nigeria. Olusegun, had while expressing dismay over which about 11,000 doctors and nurses leaving the country annually for greener pasture; says medical tourism has become a major global business [16]. Furthermore, the thing becomes worse to the extent that Nigeria losses N359.2 to medical tourism annually which is more than the N340.45 Billions allocated to the health sector in the 2018 budget [17].



THEORETICAL FRAMEWORK

This study is anchored on the Dependency Theory also known as the Underdevelopment theory/the Neo-Marxist theory/the Radical School of Thought. It came as a straightforward reaction to the Modernisation theory in elucidating why the center is developed, while the periphery is underdeveloped. The exponents of the theory include Raul Prebisch, Andre Gunder Frank, Claude Ake, Walter Rodney, Samir Amin, Frantz Fanon, among others. The theory was developed in the late 1950s under the guidance of the Director of the United Nations Economic Commission for Latin America, Raul Prebisch.

Dependency theory categorized the world into two – the Core or Centre like USA, United Kingdom, France, Germany, etc, and the Periphery or satellite which comprise the colonized and underdeveloped countries of the world like Niger, Chad, Nigeria, South Africa, to mention just a few. The theory contends that the export of capitalism by the West to other areas of the World and its consequential colonialism cum neo-colonialism is liable for the backwardness and reliance of the Third World Countries on the center. Therefore, in development studies, dependency implies a condition in which a particular country or region depends on another for assistance, survival, and advancement.

Like other Dependency theorists, Andre Gunder Frank (1976), concludes that the Periphery feed the Centre with its cheap labor and inexpensive primary goods like cocoa, crude oil, cotton, rubber, palm oil, groundnuts, etc, while the Centre stagnates and underdevelops the Periphery with its capitalist exploitation, the export of costly finished goods, unfavorable terms of trade and exploitative international politico-economic capitalist systems and institutions such as globalization, the World Bank and the IMF. All these factors separately and exclusively, have driven the backwardness of the Third World Countries and their seeming perpetual reliance on the Industrialised Capitalist States [18]. Therefore, the Dependency School of Thought recommends that the only way the Third world can attain development is to “delink” their economies from the basis of domination and exploitation and start building their nations so that they can stand with their feet.

However, the critics of the dependency theory are of the view that Dependency School of thought is so much concentrated on the exploitation of the resources of the developing nations by their ex-colonizers; that it oversights the role of corruption, bad governance, and political instability in the underdevelopment of the Third world nations.

The applicability of this theory to this study is the fact that Nigeria's health care system is underdeveloped because of the surplus extraction of Nigeria's medical experts by the capitalists to the global wealth centers.

CAUSES OF MEDICAL BRAIN DRAIN IN NIGERIA

There are complex determining factors influencing the massive migration of health professionals from Nigeria and they can be broadly categorized into two; push and pull factors.

I. PUSH FACTORS:

Push factors are associated with the country of origin and include insufficient income and miserable working

conditions. Inadequate salaries of medical professionals that will gratify their critical needs and lack of promotion at the right time compelled many to migrate and work abroad. It is obvious that Nigeria is still a country with low labor payments in which its minimum wage is ₦18, 000 about 49 US\$. If we compare this minimum wage with that of the United State of America, United Kingdom, Australia, etc, Nigerian wages have been lagging far backward from these developed nations [19]. This necessitates many to migrate to developed countries where they can be better paid for their services. Secondly, poor working condition is another major driver that pushes doctors to migrate from Nigeria. This includes having to work extra hours due to insufficient personnel and the inadequacy of diagnostic capabilities. The migration of nurses from their home country also results from dissatisfaction from work which includes constant schedule changes, work overloads due to a high number of patients and increased paperwork, unfavorable shifts, lack of appreciation by seniors, and inadequate pay package. Consequently, it becomes unavoidable for many medical experts to migrate where they would be better paid.

Thirdly, the absence of job openings and social recognition, oppressive political climate, and lack of job opportunity is another constraint that pushes medical doctors to leave Nigeria. In an interview with former Health Minister Adewole, express concern over the lack of space for many doctors and advise them to become entrepreneurs. He said "Currently, I made mention one of our hospitals that interview people and has room for less than 150, which is less than 1:5 and over one thousand people applied. He further said, the last time University College Hospital (UCH) did recruitment tests for residents into the Obstetrician and Gynecology department, over 200 persons applied for six positions. They set examinations for them and I think about 50 passed and yet only six of them were needed" [20]. Therefore, lack of job opportunity also contributes immensely to the Nigerian medical brain drain.

2. PULLFACTORS:

Pull factors are associated with the country of destination that has a strong economy and the possibility of advancement. These pull factors include better remuneration and working conditions. Better remuneration, attractive and conducive atmosphere of working place are the major pull factors that tantalizes many Nigeria's specialists. Hence, many Nigerian medical doctors prepare to migrate where their needs would be satisfied. For example, a developed country like the United Kingdom pays annual net wages of 43,732.30(US\$) while in Nigeria the annual earnings of an average Nurse working in a government institution are 5,000(US\$) [21]. Thus, higher compensations become a motivating pull factor among Nigerian Nurses.

Secondly, prospects for career development, job satisfaction, and political stability in developed countries also pull a lot of medical professionals to migrate and work there. According to Abraham Maslow's Hierarchy of human needs, security is one of the most important human needs. Therefore developed countries where there is a

socio-political environment nurses from developing nations like Nigeria where the masses are confronted with social chaos varying from tribal, economic, political, to religious crises prepared to leave the country and work outside. Thirdly, there is lack of patriotism from the angle of medical professionals whose most of them received scholarships from the Nigerian government and leave the country eventually. It is disheartening to leave your relatives without doctors that would monitor their health challenges to go and provide services to those that have no such challenges. Therefore what they ought to do is to stay and confront the hassles in order to be rectified. Thus, lack of patriotism from the medical professionals also contributed immensely to their migration.

THE IMPLICATIONS OF MEDICAL BRAIN DRAIN IN NIGERIA'S HEALTH-CARE SYSTEM

The effects of Nigeria's medical migration is spinning out of control as the health care system of the country is worsening due to inadequate medical specialists. Thus, the narrative that the medical brain drain contributed immensely to the decline of Nigeria's health care system is persuasive. To be convinced by this assertion, this paper argues that medical brain drain was responsible for high rate of maternal death in Nigeria due to lack of specialists that would take care of the patients. The World Health Organization (WHO) specifically commends a nurse to a people ratio of 700, but as of 2014, Nigeria has less than 150,000 registered nurses to cater to an estimated over 160 million population, giving an average nurse population ratio of 1 to 1,066 people [22]. Therefore going by WHO recommendation, Nigeria is not on track to meet the very low benchmark of 2.5 doctors, nurses and, midwives per 1,000 people. It is regrettable Nigeria as the giant of Africa, to be ranked among the worst countries in regards to maternal health outcomes; although it represents 2% of the global population, it disproportionately contributes to nearly 10% of global maternal deaths [23]. Due to its high maternal mortality ratio, Nigeria also ranks as the leading contributor to all maternal death figures in Sub-Saharan Africa despite its relatively high human and material resources, as well as gross domestic product [24]. Therefore, medical brain drain contributes to the many maternal death in Nigeria.

Secondly, medical brain drain has caused ineffective service delivery in Nigeria's health sector. As a result of the migration of medical specialists, Nigeria's health care system was not standard in the sense that patients are not getting effective services that will cure their illness at the right time. The consequences of the medical brain drain have forced the workers in the sector to working for extra hours due to insufficient personnel and inadequacy of diagnostic capabilities, to mention just a few. Hence, ineffective service delivery becomes inevitable. Furthermore, the consistent schedule changes, overload works as a result of a high number of patients and high paperwork, unfavorable workplace transfer, lack of appreciation by seniors, and insufficient pay are also factors responsible for the ineffective service delivery in Nigeria's health care system. This, in turn, adds stress and dissatisfaction among those remaining. Thus, necessitating many to traveling abroad for medical attention from health experts.

Thirdly, the implication of medical migration on Nigeria's

economy cannot be overemphasized. Nigeria's politicians whose enhancing the country's health system is their responsibility usually travel abroad for their medical care. The usual destinations are Europe, North America and, Asia, United Arab Emirates, etc. This of course has negatively affected the country's economy because of the huge amount of money that Nigeria spends on Medical tourism annually. Through an average of 9,000 medical trips occurring monthly from Nigeria to other countries, the nation losses N359.2 billion (1.35 billion dollars) to medical tourism annually which is more than the N340.45 billion allocated to the health sector in the 2018 budget where India being the major beneficiary of 500 visits monthly [25]. Many Nigerians who travel out of the country for their medical needs often have to go back-monthly for check-ups and sometimes for corrective surgery. So imagine if all this huge amount of money will be spent in Nigeria's health sector how effective it will be? Therefore, medical brain drain contribute to the economic deterioration in Nigeria.

CONCLUSION

The medical brain drain has contributed immensely to the decrease and ineffectiveness of Nigeria's Health Care System (HCS). It has been revealed that part of the causes of this problem include: poor payment system, poor condition of service, lack of security, etc, which caused retrogressiveness in the Health care system and Nigeria's economy. Therefore, there is need to reverse the medical brain drain and provide comparable consultant and personal chances for our most talented people at home in Nigeria. Only in this way will stay in Nigeria or payback - not out of a sense of duty or sacrifice, or feeling, but instead because their home country is a rewarding and fascinating place to live and work. We must make Nigeria overpowering to those who pursue the Health profession, by doing this, soon Nigeria's health care system will be among the most standard in the world and the medical brain drain in Nigeria will become history as it becomes in Thailand, Ireland, and, China, among others.

Recommendations

Base on the foregoing discussion, the following recommendations are provided:

- i. To reverse the medical brain drain, Nigeria needs to address the structural, political, and economic problems that lead to the brain drain and demonstrate foresight and dedication to enhance the living conditions of the people to succeed in generating a brain gain through fascinating back medical experts and offering generous research funding and monetary incentives as well as services and assistance to them.
- ii. All Nigerian government officials should be restrained from medical tourism and create a conducive health care system that will ensure adequate and satisfactory services and system of payment that includes payment on time, due process in promotion, allowances, pensions, providing financial and other incentives to stay, institutional capacity-building that promotes career development should be fostered, along with mentorship opportunities as well as efforts to improve working conditions; providing educational benefits for their children;
- iii. Nigeria should create employment opportunities and reduce

poverty and provide the needed infrastructures such as good roads and transport systems, affordable and qualitative education, water supply, security, stable energy in addition to a good health care system.

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